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iMAP/DFS COVID-19

Situation Analysis

Crisis type: Epidemic

Period: June 30 - September 30 2020



BANGLADESH

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Executive Summary

- The Government of Bangladesh moved quickly to impose significant containment measures after the first in-country cases of COVID-19 were confirmed on March 8. The national lock-down initiated on March 26 was followed by stringent guidelines and restrictions on humanitarian activities in Cox's Bazar.
- For several months activities were limited to those deemed "critical" (this included health care, food and cooking gas distribution, and WASH). Even these activities were still impacted as aid agencies attempted to limit the humanitarian footprint to comply with social distancing and hygiene measures. Many other activities such as livelihood programs, health awareness campaigns and some protection services were curtailed or suspended. Education centers, child and women-friendly spaces, community centers and most markets were closed.
- The impact of containment measures on the camp population's safety and well-being has been substantial and wide-ranging in a context where 94% of households were considered highly or moderately vulnerable before the crises. Loss of income, lack of access to markets and issues with food distributions have contributed to rising food insecurity with many households resorting to negative coping mechanisms to meet their basic needs. This includes families reducing food portions, missing meals, selling assets or buying food on credit. The situation has also had both acute and long-term effects on the host community. Many people have lost their jobs and the crisis has pushed many families below the poverty line.
- Conditions in Cox's Bazar are extremely congested with many families living in one room tents. The imposition of movement restrictions and closure of education centers and other public spaces put increased pressure on households. Levels of stress and anxiety brought on by the loss of income, food insecurity, and a lack of access to normal support networks have driven up feelings of frustration and despair. This has manifested into violence in the home with women and children often the target. Reduced protection monitoring and the suspension of some services has made it more difficult for those affected to seek help or for cases to make it into and through the referral system. Incidences of child marriage and child labor are rising. Increased criminality and decreased social cohesion are significant factors contributing to a higher risk of sexual and gender-based violence (SGBV) and people trafficking. Education Centers remain closed and efforts to provide distance learning in the home are challenged by the crowded conditions, low educational levels amongst parents and caregivers and the lack of support and training required by volunteers and teachers to switch to a new modality. It is feared that if schools do open, many children, especially girls, will not return.
- As is often the case, the poorest and most vulnerable have been impacted most. The worst affected include female-headed households and households with high numbers of dependents but only one income source. Close to half of the female-headed households in Cox's Bazar Municipality were found to be highly vulnerable following the crisis; in contrast, a quarter of the male-headed households are in the same position. The easing of restrictions and resumption of humanitarian activities will alleviate some of this impact. However, there is still a gap in services compared to pre-crisis levels, and the long-term effects on health and well-being are unknown.
- Identified cases of COVID-19 have been rising significantly recently (151 cases in the last 4 weeks of September were confirmed compared to 26 cases in the previous 4 weeks of August). The response, including testing and isolation, has been undermined by fear and mistrust in the Rohingya community. Consultations for acute respiratory infections (ARI) dropped dramatically during the first three months of the crisis and this was a contributing factor to the initial lack of testing. Providing relevant COVID-19 messaging to the community has been challenging. Lack of phone and internet services have restricted the available information channels.

Significant efforts are being put into communication with communities through community health volunteers and by pushing messaging out via many humanitarian activities currently being implemented. The recent lifting of mobile phone and internet restrictions should facilitate this. However, there is still work to be done to strengthen compliance and encourage those with respiratory illnesses to seek medical support. Testing has increased, with over 5,000 tests conducted in September, double the total number of tests conducted up to the end of August. Since June, measures have been in place for temperature screening and sanitization at all 19 entry points to the camps.

- Moving forward, humanitarian agencies will continue adapting to the new normal of providing services following COVID-19 prevention procedures. Hopefully efforts will be successful in increasing community compliance with COVID-19 measures, especially testing, isolation, and quarantine. Lack of education services remains a concern, although opening education centers and women and child-friendly spaces can increase the risk of COVID-19 transmission (though this is difficult to quantify). Increased COVID-19 cases could result in expanded restrictions (mirroring the "red zone" declaration for Cox's Bazar in June) and this remains a significant risk with all the attendant impacts that such limits would bring. Finally, the prevalence of protection issues remains a central focus of continued humanitarian efforts.

Table of Contents

Executive Summary	2
Table of Contents	3
COVID-19 Epidemic Overview	5
Cox's Bazar	5
The trajectory of COVID-19 in Cox's bazar	6
Future risks	7
COVID-19 Containment Measures	7
COVID-19 containment measures at national level	8
Containment measures in Cox Bazar	8
Challenges and impact of containment measures	8
Testing ramped up after a slow start	8
Containment measures eased as rains arrive	9
Current situation	9
Communication and Information	10
Information Channels and Means	10
Information Challenges	11
Information Needs	11
Overview of impact and humanitarian conditions	11
Crisis Impact	12
Worsening of humanitarian conditions are driving adoption of negative coping mechanisms	
Livelihoods	13
Food Security	14
Protection	17
Education	19
Information gaps and challenges	20
COVID-19 Infection rate	20
Livelihoods and Food Security	21
Impact of Restrictions on Humanitarian Activities	21
Protection and Education	21
Annex – About this report	22
Contact	26

COVID-19 Epidemic Overview

The first cases of COVID-19 were confirmed in Bangladesh on March 8 ([Reuters](#) 08/03/20). The Government declared a general holiday from March 26 to May 30, closing government offices and non-essential businesses, and restricting movements. These measures have now lifted, but economic activities remain subdued ([UN Country Team in Bangladesh](#) 16/09/2020). The COVID-19 crisis has had enormous repercussions for poverty in Bangladesh. The country will have 16.4 million new poor in 2020 (approximately 10% of the population), according to a study by the Bangladesh Institute of Development Studies (BIDS). Some simulations estimate that the pandemic could result in the entry or re-entry of as many as 42 million people (26% of the population) into poverty in Bangladesh, undoing the decade-long gains in poverty reduction. ([UNCT](#), 16/09/2020).

The COVID-19 caseload has declined since a second peak in mid-August (rolling 7-day average¹ reached 2,832 cases on August 12), having previously peaked on July 2 (at 3,811 cases). By September 28 the 7-day rolling average was down to 1,420, from 2,189 at the beginning of the month ([John Hopkins University](#) 4/10/2020). Therefore, the caseload is still above one third (37%) of its peak and its containment remains a significant challenge.

The pandemic's impact has created a multidimensional crisis increasing immediate humanitarian needs while also exacerbating structural inequalities and pre-existing barriers in access to services, aggravating vulnerabilities in the country's governance and administrative systems. Many who were struggling to maintain minimum livelihood standards have quickly slid back into poverty. The pandemic's impacts are acutely felt by those underserved by national social protection programs, with knock-on effects on human development across society. For example, school closures affect nearly three million ultra-poor, primary school children enrolled in Government school-feeding programs ([UNCT](#), 16/09/2020). As Bangladesh battles to recover, policy decisions and the social and economic impacts of the fight against the virus will also affect the situation for Rohingya refugees in Cox's Bazar.

Figure 1: Total tests, confirmed cases and deaths for Bangladesh (WHO situation reports)

	27-Apr	25-May	29-Jun	27-Jul	31-Aug	28-Sep
Total # of tests conducted	50,401	235,014	751,034	1,124,417	1,550,203	1,921,382
Total # of confirmed cases	5,913	35,585	141,801	226,225	312,996	360,555
Total deaths (COVID-19)	152	501	1,782	2,965	4,281	5,193

Cox's Bazar

Cox's Bazar is one of the country's poorest and most vulnerable districts, with a total Bangladeshi population of 2,650,000 and it is host to some 860,000 Rohingya refugees. Cox's Bazar witnessed its first official case of COVID-19 on March 24 and the first casualty on April 24, 2020 ([WFP](#) 07/09/2020). It was not until May 14 that the

¹ A 7-day rolling average computes the average of COVID-19 cases for the last 7 calendar days.

first confirmed case of COVID-19 was reported in the refugee camps but testing until then had been limited with only 80 tests in total carried out by May 12 ([Independent Diplomat](#) 27/07/2020, [WHO](#) 12/05/2020).

Figure 2: Rohingya and Bangladeshi Population Figures in CXB

Rohingya Refugees: 860,000 (48.4% Male, 51.6% Female)

Bangladeshi Community: 2.65 Million (51.1% Male, 48.9% Female)

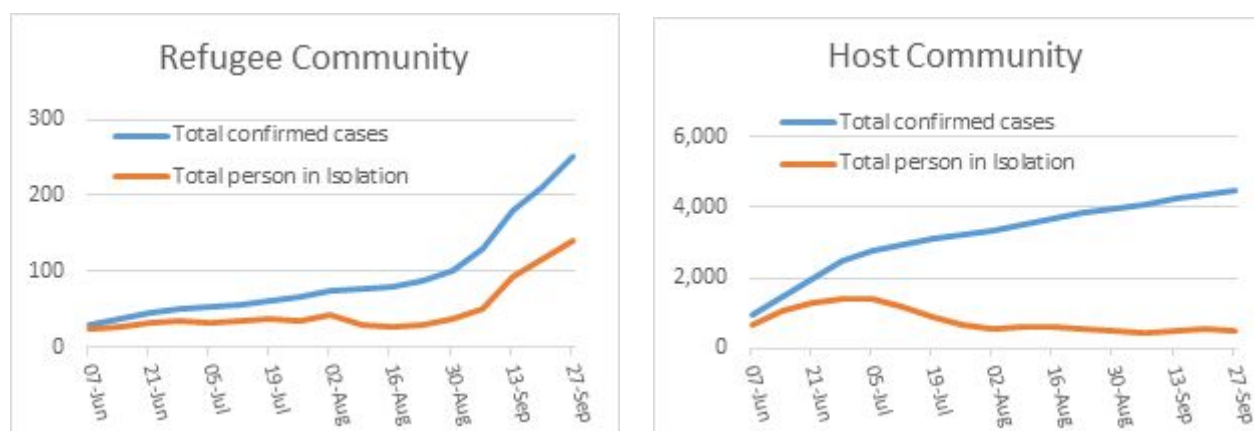
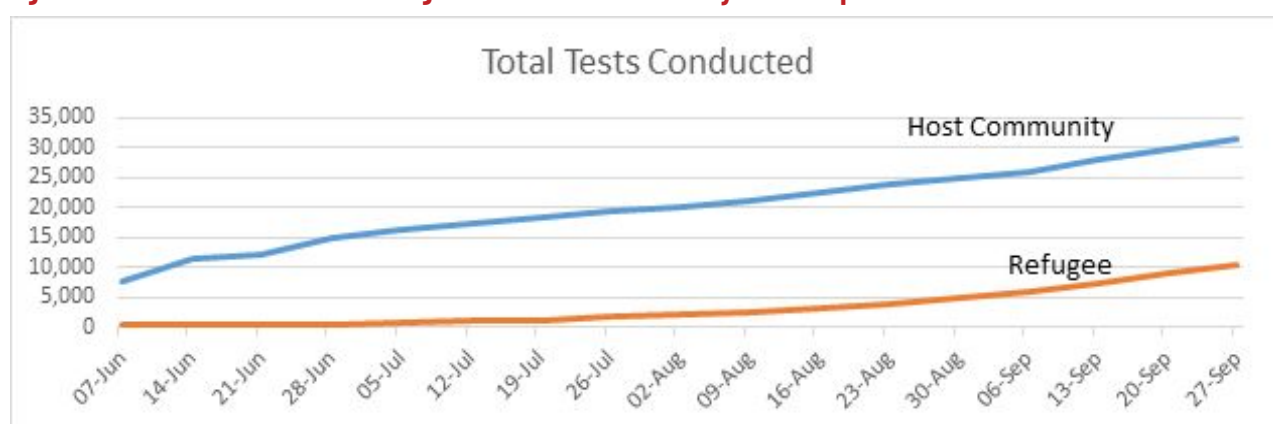
The refugee camps of Cox's Bazar are the largest and most densely populated on Earth. A majority of families share one-room shelters, with limited access to water, sanitation and basic hygiene materials like soap, conditions that would make containment measures such as social distancing very difficult to implement. In response to the looming threat posed by COVID-19, the Government of Bangladesh imposed a total lockdown on the Cox's Bazar district. From April 8, the Government forbade anyone from leaving or entering the district and blocked access to the camps for most humanitarian aid workers (with the exception of government aid workers) ([Independent Diplomat](#) 27/07/2020). Only humanitarian operations identified as "critical services and assistance" were permitted to continue and these were to be delivered using "all possible preventative measures". Critical services included health and nutrition facilities and services, food and cooking gas distribution, water, sanitation and hygiene activities ([RRRC](#), 08/04/2020).

The impact on both the refugee population and the host community has been catastrophic. The Rohingya refugee population is entirely reliant on humanitarian assistance. Travel restrictions driven by public health imperatives and global supply constraints still present severe challenges to the provision of that aid.

The District Administration predicts that more than 700,000 people in Cox's Bazar District may have become jobless because of Government restrictions related to COVID-19, based upon poverty and vulnerability estimates for the Bangladeshi population. The loss of livelihoods, coupled with decreased access to the local markets, has disrupted economic activity and increased the support needs of Bangladeshis in the District. This is especially true for the most marginalized and vulnerable, such as persons living with disabilities, households headed by women, and the elderly ([IOM](#) 22/07/2020).

The trajectory of COVID-19 in Cox's bazar

Available data shows an initial rapid increase in host community cases, which has since levelled off to a steady rise with a relatively stable number of patients in isolation. It took much longer for the refugee population case numbers to rise, although the lack of testing makes any comparison difficult. The number of confirmed cases has escalated since the beginning of September, averaging at 38 new cases per week during the month (compared to 5 cases per week over the period June 1 to August 30) and with the number of patients in isolation rising from 29 at the beginning of September to 140 by September 28 ([WHO situation reports](#) June to September 2020).

Figure 3 Cox Bazar total confirmed COVID-19 cases and number of persons in isolation**Figure 4 Total tests conducted for refugees and the host community June – September 2020**

Future risks

Current actions that aim to control the spread of COVID-19 include the containment measures still in place, significant efforts to strengthen COVID-19 information communication, and increasing testing capacity. It is hoped that these actions will increase trust for those who are sick to come forward to get tested, allow quicker identification of new cases, and foster better compliance with isolation and quarantine procedures. These factors would then help push down the COVID-19 rate of infection. However, the conditions within the refugee camps are extremely cramped, and there can be many reasons for non-compliance with containment measures or reluctance to visit health centers for testing.

If permitted, humanitarian activities maybe expanded to mitigate some of these issues, although as is currently the case, all activities are required to minimize COVID-19 risk, which makes their effective implementation more challenging.

If measures such as the reopening of education centers are taken, this will positively impact children's mental health and well-being and present further opportunities for providing COVID-19 prevention information. However, bringing children from multiple households together also leads to an increased risk of transmission.

COVID-19 Containment Measures

The Government of Bangladesh moved quickly to impose significant containment measures after the first in-country cases of COVID-19 were confirmed on March 8. The national lockdown that was initiated on March 26 was followed by stringent guidelines on restrictions on humanitarian activities in Cox's Bazar. For several months' activities were limited to those deemed "critical" (this included health care, food and cooking gas distribution and WASH). Still, even these activities were impacted as aid agencies attempted to limit the humanitarian footprint in the camps and take necessary precautions to prevent the spread of COVID-19.

Despite the measures COVID-19 cases steadily rose within the camps. With the extremely crowded conditions and the general reluctance of the refugee population to get tested or seek health care this was not surprising. As measures are now beginning to be eased (additional humanitarian services have been allowed to restart, movement restrictions have been reduced, internet connections have been restored) it is hoped that the negative impact will lessen. Still, the longer-term effect on health and well-being is not known. Besides, unless COVID-19 cases reduce there is the possibility that stricter containment measures will be reintroduced.

COVID-19 containment measures at national level

From the outset of the pandemic the Government of Bangladesh followed similar policies to much of the rest of the world by introducing stringent containment measures aimed at stopping the spread of the virus; however, these measures also had a significant negative impact on both livelihoods and the provision of basic services. The first cases of COVID-19 were confirmed in Bangladesh on March 8 ([Reuters](#) 08/03/2020) and as case numbers rose, containment measures were introduced including the closing of all educational institutions and suspension of all academic activities on March 17 ([IFRC](#) 17/07/2020). A nationwide lockdown (initiated on March 26) was soon followed by the suspension of all domestic and most international flights ([IFRC](#) 15/07/2020, [IOM](#) 22/07/2020).

Containment measures in Cox Bazar

Measures affecting the refugee-hosting district of Cox Bazar were also stark. From April 8, 2020, the Government forbade anyone from leaving or entering the Cox Bazar district, and blocked access to the camps for most humanitarian aid workers. Roadblocks stopped an estimated 80 percent of aid workers from entering and leaving the district ([Independent Diplomat](#) 27/07/2020). Only humanitarian operations identified as "critical services and assistance" were permitted to continue and these were to be delivered using "all possible preventative measures". Critical services included health and nutrition facilities and services, food and cooking gas distribution, water, sanitation and hygiene activities ([RRRC](#), 08/04/2020). Humanitarian organizations adapted programming modalities for essential services to minimize humanitarian footprint and reduce the chance of spreading the virus. For example, food distributions were changed from twice monthly to monthly ([IOM](#) 31/07/2020). "Non-essential" interventions were scaled down or suspended including activities such as health education and community mobilization for TB awareness ([WHO](#) 26/08/2020).

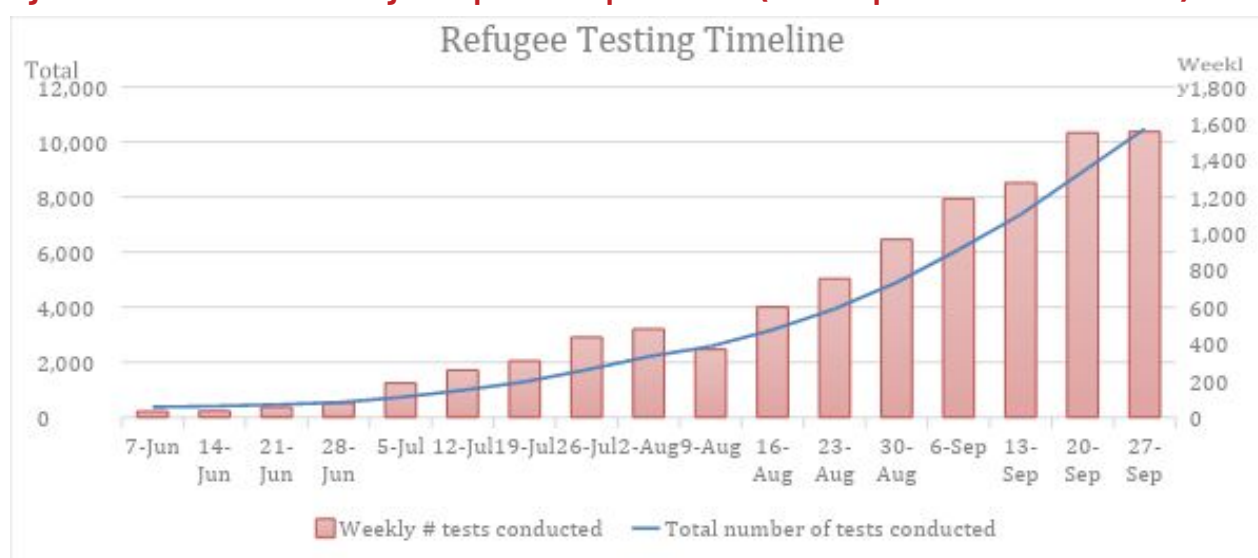
Challenges and impact of containment measures

In the early days of the pandemic, it became apparent that while physical distancing was being mandated by governments across the world, it would be particularly challenging in Cox's Bazar where households live in such proximity to one another ([IOM 2020/09/11](#)). Moreover, containment measures like physical distancing and isolation due to COVID-19 could contribute to already existing structural problems such as overcrowding as the shelters are small and shared by an average of 5-6 family members ([IOM 2020/07/30](#)). There was also a significant negative impact on the accessibility and availability of many services that the camp populations rely on ([IOM 2020/07/31](#)).

Testing ramped up after a slow start

Testing for Rohingya refugees was initially limited, with only 460 tests conducted by June 24 ([WHO 26/06/2020](#)), three months after the initial lockdown had been introduced. There was a fear and unwillingness to be admitted to isolation and treatment facilities due to the lack of trust in both the health system and the camp authorities; this was compounded by misinformation and rumors surrounding COVID-19 testing and isolation quarantine among the FDMN/Rohingya. These factors were significant contributors to the low consent rate for testing ([ACAPS, 07/07/2020](#), [WHO 26/06/2020](#)).

Figure 5 Timeline of COVID-19 testing in camps June – September 2020 (WHO Sitreps 10/06/2020 – 27/09/2020)



The number of COVID-19 tests carried out in the camps rose steadily from mid-July onwards with refugee community health volunteers engaging in community-based surveillance and providing targeted counseling on testing and related issues to refugees identified with COVID-like symptoms ([UNHCR, 27/08/2020](#)). 1,550 tests were completed in the week 17 – 24 September.

Containment measures eased as rains arrive

Cases in Cox's Bazar continued to rise despite the restrictions. In early June Cox's Bazar municipality was designated as a 'Red Zone' following a decision of the District Committee on Coronavirus Prevention. The measures were implemented from June 6 to June 20 due to the high rate of COVID-19 cases. The area was under a strict lockdown with markets only open on Sundays and Thursdays. ([WFP, 01/07/2020](#), [UNHCR 24/06/2020](#)). However, as the monsoon season started (in late June) there was a limited easing of some lockdown measures in Cox's Bazar District. Aid agencies were asked to follow guidelines set by Bangladesh's Refugee Relief and Repatriation

Commissioner (RRRC) on activities that are permitted in the camps. The scope of permissible activities was updated to include limited registration activities, expanded monsoon response and repair work. ([UNHCR](#) 2020/08/10, [UNHCR](#) 2020/08/16)

In June, the government authorities have introduced a zonal system to contain the virus's transmission amidst heavy rainfall. Temperature screeners, educators and WASH volunteers work together as a team to screen, educate, refer and sensitize the Rohingya, host community and humanitarian workers. This includes temperature screening at all 19 points of entry to the camps.

The suspected and confirmed COVID-19 cases in the refugee camps are being referred to isolation and in quarantine facilities staffed with trained health care workers ([WHO](#) 2020/09/03, [BDRCS](#) 2020/08/03). Even so, people have distrust with the authorities and are unwilling to be admitted to these facilities. This lack of trust stems from a combination of negative experiences with health staff pre-COVID, collective memory of ill treatment in hospitals in Myanmar, an unwillingness to be separated from their families, reports of mistreatment from those first suspected of contracting COVID-19 in the camps, and a misunderstanding of what is provided at these facilities ([ACAPS](#) 2020/07/05).

Current situation

The trend for the number of COVID-19 cases identified has stabilized over the last weeks (ending September 30) even with an increase in the number of tests being carried out (see Fig 6). If the number of new COVID-19 cases does not rise significantly, it would be hoped that further containment measures will not be introduced, and perhaps some further easing of restrictions may be possible.

Figure 6. Weekly count of tests and positive cases for the last 5 weeks

Week ending:	30-Aug	06-Sep	13-Sep	20-Sep	27-Sep
Weekly # tests conducted	969	1,191	1,277	1,550	1,556
New cases	13	29	50	32	40

With the increasing efforts and resources being put into communicating COVID-19 information and the continued development of services in line with government directives it can be hoped that these measures will positively affect compliance by the camps population with COVID-19 physical distancing, isolation and quarantine procedures. Monsoon rains that have provided an additional hazard and aggravating factor should start to ease in October and the concern is that the longer containment measures remain in place, the more compliance rates may drop.

Communication and Information

Initial efforts to provide information on COVID-19 were hampered by lockdown measures that limited the humanitarian footprint in Cox's Bazar. Information dissemination used loud-speakers and radio supported by messaging through mosques and those humanitarian services that were permitted to continue. As the situation progressed, community health volunteers were trained to provide outreach with the aid of printed, audio and video materials in Burmese, Bangla and English (providing information in the right language was another barrier).

Distrust of the authorities, a lack of access to the internet to source information and the spread of disinformation and rumors all had a negative impact on the compliance with containment measures with the lack of testing and willingness to quarantine being of particular concern. The number of those self-referring for acute respiratory Infections dropped by approximately two thirds over the first three months of the crises.

More recently, the number of community volunteers has increased, and more consistent messaging has been provided through trusted sources such as mosques. This is beginning to counteract the impact of misinformation and to some extent explain and reassure what and why testing, quarantine and isolation measures are necessary.

The restoration of mobile and internet services at the end of August could provide an additional channel for COVID-19 information provision. Without any announcement on the reopening of education centers, there remains a gap in providing relevant and appropriate information to children who remain ignorant and fearful of the virus.

Information Channels and Means

Hygiene promotion efforts have been supported by messaging on COVID-19 through household-level visits, by megaphone, audio/USB sticks and via mosques. Community-based protection partners, and the refugee community continue to conduct awareness-raising sessions, hygiene promotion and safe group radio listening activities in the refugee settlements ([UNHCR](#) 2020/08/10). Trained community health volunteers are working with the community and religious leaders to communicate important lifesaving and prevention messages on health and hygiene to camp and host communities. The volunteers are conducting this outreach with the aid of printed, audio and video materials in Burmese, Bangla and English languages. They also disseminate hygiene messages through community-based messaging, community engagement, discussions and community sessions ([BDRCS](#) 2020/08/03). Burmese Language Instructors (BLIs) and educational facilitators disseminated COVID-19 awareness messaging in the camps through audio clips to children, parents, community members and caregivers. In the host communities, volunteers and facilitators distributed COVID-19 awareness leaflets and COVID-19 awareness messages to children, parents, community members and caregivers ([ISCG](#) 2020/09/09).

Information Challenges

The lack of internet communication leaves the population without access to medical guidance and severely hampers the community's ability to communicate to the outside world ([Independent Diplomat](#) 2020/07/27). Sharing information about COVID-19 and raising awareness among communities has been crucial to MSF's response, but doing so using social media or SMS messaging has been hampered by restrictions on mobile network availability in and around the camps ([MSF](#) 2020/08/25).

The stigma and pervasive misinformation related to COVID-19 have the potential to exacerbate existing health problems and impede efforts to mitigate the disease outbreak. These factors can dissuade people who may be infected from seeking care or cause households to hide sick family members to avoid discrimination, especially for minorities and marginalized groups ([IOM](#) 2020/07/22). Total consultations dropped by over 50% between March and July, Acute Respiratory Infections (ARI) accounted for only 14.3% of the total consultations at the beginning of July down from 26.4% at the end of March. This two third reduction of total ARI consultations indicates changes in specific health seeking behavior of refugee population ([WHO](#) 13/07/2020).

Many misconceptions and myths are spread regarding COVID-19, including that the pandemic is a punishment from God and the spread of rumors and misinformation have created fears among the Rohingya. As a result, many are avoiding health services including immunization, maternal health and family planning which is reflected in several health outcomes. The decrease in immunization rates is worrying as it presents a risk for the emergence of vaccine-preventable diseases (VPD) ([CARE](#) 2020/07/01 [WHO](#) 12/08/2020). Children are scared and struggling to access the information of COVID-19 they need. The information that is available is not age-appropriate and understandable, especially for younger children. With child protection facilities and education centers now closed, it is also more difficult to reach children ([UNICEF](#) 2020/09/17).

Information Needs

The main challenge is ensuring the provision of information on COVID-19 quickly, through trusted sources (such as religious leaders) and in the right language, media, and format appropriate for the age and gender of the recipient. It is also crucial to address and correct rumors and misinformation quickly through rumor tracking mechanisms. The effective dissemination of messages to all calls for the broad engagement of different groups, including religious leaders and refugees participating in camp governance structures and non-formal mechanisms. Community awareness and community-led approaches are key to mitigating the impact of the COVID-19 pandemic in Cox's Bazar. Clear, understandable, and readily available information and multimedia messages are needed in Bangla, Burmese, and Rohingya, including on how to protect individuals and communities, what to do in case of sickness, on what to expect from the health response, such as the isolation and quarantine processes ([IOM](#) 22/07/2020).

Overview of impact and humanitarian conditions

Cox's Bazar is one of the country's poorest and most vulnerable districts and is highly susceptible to recurrent climatic shocks. The district is home to a total Bangladeshi population of 2,650,000, of whom approximately 33 percent lived below the poverty line (figures from 2017) ([IOM](#) 22/07/2020, [WFP](#) 07/09/2020).

The humanitarian situation in Cox's Bazar has deteriorated since the beginning of the COVID-19 crisis in March 2020. However, it is essential to understand that there were significant needs and vulnerabilities within the district for both Rohingya refugees and the host community before the pandemic began. The camps are among the most densely populated places on Earth, with the overcrowding and unhygienic conditions increasing the potential for the rapid spread of communicable diseases ([IOM](#) 22/07/2020).

Crisis Impact

COVID-19 containment measures had a wide-ranging impact on the refugee population and camp services including:

- **A loss of livelihoods** and income due to lock-down and movement conditions.
- **Stress on health services** with the need to implement COVID-19 prevention measures such as the use of PPE, cleaning and hygiene and social distancing.
- **Reduction in health services**, including the pausing or scaling down of vaccination and TB awareness campaigns.
- **Disruption** to other **humanitarian services** such as **WASH** and **food distribution** due to changes in operating procedures to minimize the humanitarian footprint in the camps and mitigate against the spread of COVID-19. In addition, staffing challenges due to movement restrictions and the closure of airports also reduced humanitarian capacity.
- **Suspension of humanitarian services deemed non-essential**, including the **closure of education centers** and many **women-friendly** and **child-friendly spaces**. Education centers still remain closed at time of writing (September 30 2020).
- **An increase in protection issues and risks** with families forced to spend prolonged periods in single room tents with limited access to normal social support services.
- **Fear of COVID-19** and lack of trust/understanding in testing, isolation and quarantine procedures leading to those feeling sick not seeking health services.

The situation was further worsened by the onset of the monsoon season in June which caused increased difficulties in terms of the distribution and collection of relief items (primarily food). Wind, heavy rains and flooding has resulted in damage to shelters and other infrastructure impacting over one thousand households ([ECHO](#) 19/08/2020, [UNHCR](#) 16/08/2020). For the host community, the impact on livelihoods was huge with the District Administration predicting that more than 700,000 people in Cox's Bazar District (a quarter of the population) may have become jobless due to containment measures. Households faced similar challenges in terms of increased protection issues, closure of schools and more limited access to health services.

Worsening of humanitarian conditions are driving adoption of negative coping mechanisms

With the negative impact on people and services, humanitarian conditions worsened, and this also triggered an increase in households and individuals relying on negative coping mechanisms. Issues of particular concern include:

- **Increasing food insecurity** due to problems related to food distributions, lack of access to markets and the reduction of household purchasing power.
- **Households are relying on mostly negative coping mechanisms** such as eating less, reducing the number of meals per day, relying on support from family or friends or selling assets/going into debt to meet food needs.
- **Delivery of education services for the camps is still extremely challenging** and mostly limited to the distribution of home learning materials and parent/caregiver instruction. Without internet and language

appropriate television Rohingya children are struggling to continue their education. Conversely for host communities, Government initiatives have provided a range of services for distance learning.

- **Protection issues are becoming more prevalent** including intimate partner violence, inter and intra-communal violence, child marriage and child labor. Children are also witness to and the subject of violence in the home. Criminality and security concerns are also rising, heightening the risks of SGBV and people trafficking.
- **Lack of access to normal support mechanisms especially for children and women.** Because of movement restrictions and without access to education centers, children do not meet with their peer group or take part in normal recreational activities. When spouses are confined to the home its is harder for women to access SGBV services.
- Overall the situation is driving a **decrease in mental well-being** due to the stress of living in confined spaces, increased worries over access to food and livelihoods, increased violence and conflict, a lack of future prospects (with education services remaining closed) and a fear of COVID-19 both as an illness and of the resulting measures such as isolation and quarantine as well as the stigma attached with having COVID-19.
- These issues are prevalent amongst both the refugee and host community, although as highlighted earlier, with less access to humanitarian services, livelihood issues are of particular concern to the host community with the poorest and most disadvantaged households affected most. This is succinctly summarized in the Immediate Socio-economic Response Plan: "The COVID-19 pandemic is impacting the poorest and most vulnerable in Bangladesh with the greatest severity. The extreme and moderate poor were hardest hit by the two-month-long general lock-down, suffering an average of three-quarters loss in income and the attendant consequences in access to health care and nutrition along with disproportionate impacts on education, protection and employment" ([UNCT](#) 16/09/2020).

The following analysis looks more closely at four sectors. Livelihoods and Food Security, sectors which are intrinsically linked by the impact of COVID-19 containment measures; Protection, which is of particular concern as the pandemic has put extreme pressure on both households and humanitarian services; and finally Education, both as consideration of the longer term impact of the crisis on children and families and to highlight how lack of such a service has wider implications especially in terms of driving protection issues and a reliance on negative coping mechanisms.

Livelihoods

The vast majority of Rohingya refugees were already almost entirely dependent on humanitarian aid to meet their basic needs before the COVID-19 pandemic with 94% of the population considered highly or moderately vulnerable. Also, Cox's Bazar is one of the poorest districts in Bangladesh.

COVID-19 containment measures had a catastrophic impact on livelihoods and access to income-generating activities for refugees and host communities. Households with high numbers of dependents and only one income are amongst those most severely affected. Female-headed households are also disproportionately affected with close to half of the female-headed households in Cox's Bazar Municipality found to be highly vulnerable following the crisis; in contrast, a quarter of the male-headed households are in the same position.

Loss of income led many households to adopt negative coping strategies, including selling assets, taking on debts to pay for food, reprioritizing products such as soap or sanitary pads, reducing food intake (both quantity and quality) and an increase in incidents of child marriage. The security situation remains tense with reports indicating an increase in criminality and the formation of vigilante groups as refugees join gangs as a coping mechanism following the loss of livelihoods.

As restrictions ease, many will have to balance the need to return to work and the risk of being infected with COVID-19. Many households will now have fallen below the poverty line and are even more vulnerable to further shocks (such as the reintroduction of lockdown or stringent movement restrictions).

Catastrophic impact of COVID-19 Containment measures on livelihoods

COVID-19 containment measures had a catastrophic effect on the livelihoods of both refugees and the host community, threatening to push even higher numbers into poverty and driving many to adopt negative coping mechanisms.

The vast majority of Rohingya refugees were already almost entirely dependent on humanitarian aid before the COVID-19 pandemic with 94% of Rohingya refugees considered highly or moderately vulnerable and in need of humanitarian assistance to meet their basic needs ([WFP](#) 04/2020). The limited income generation activities available pre-COVID have been disrupted by movement restrictions, market closures, and a reduction in humanitarian-led income-generating activities, such as volunteering for NGOs or participating in Cash for Work programs ([ACAPS](#) 05/07/2020). Cox's Bazar is one of the country's poorest and most vulnerable districts and the District Administration predicts that more than 700,000 people in Cox's Bazar District (a quarter of the population) may have become jobless because of Government restrictions related to COVID-19 ([IOM](#) 22/07/2020). Household income levels have been significantly impacted by livelihood disruptions, with families generally losing between BDT 1,000 to BDT 3,000 a week (12 to 35 USD). Food shortages were mentioned as concerns by refugee households and shortages of medication by the host community households ([WFP](#) 01/07/2020).

The poorest have been impacted the hardest

The lockdown has affected nearly everyone, but its impacts are particularly acute for people in poverty and daily wage earners. Those who were more economically vulnerable pre-COVID-19, such as single female-headed households, are more vulnerable to the secondary economic impacts. They are less likely to have savings or assets that could help them absorb the initial shock and mitigate the long-term negative impacts (([UNDP](#) 31/07/2020, [ACAPS](#) 07/07/2020). Close to half of the female-headed households in Cox's Bazar Municipality were found to be highly vulnerable following the crisis, in contrast, a quarter of the male-headed households are in the same position ([WFP](#) 07/09/2020). Households with a high number of dependents (dependency ratio) are also more likely to be highly vulnerable, both in pre-crisis and under the current situation. This indicates that households with more earning members and alternative Income Generating Activities (IGA's) would be more protected from the harsher effects of the lockdown as opposed to a family with one earning member and one income source. However 72 percent of the households in Cox's Bazar Municipality report having only one income source and one earning member ([WFP](#) 07/09/2020).

Loss of livelihoods driving increased violence, criminality and the adoption of negative coping mechanisms

Loss of livelihoods is a contributing factor to the increase in violence reported across the camps over the past six months with the security situation in the southern camps remains particularly tense. Refugees have begun forming vigilante groups to counter these criminal gangs, while an increasing number of refugees have reportedly joined gangs as a coping mechanism, especially considering the lack of livelihood opportunities as a result of COVID-19 restrictions ([UNHCR](#), 11/08/2020).

For Cox's Bazar host community, 9 out of 10 households resorted to livelihood-based coping strategies to meet both food and non-food needs. Spending savings and buying food on credit were the widely adopted strategies, reported by 63 and 44 percent of households, respectively ([WFP](#) 07/09/2020). People are now facing a food crisis, and this may influence them to deprioritize hygiene materials like soap and sanitary pads ([WaterAid](#) 05/08/2020). Many are forced to choose between health and livelihoods. If they prioritize livelihoods, they risk getting sick and not being able to afford appropriate care, which in turn would also lead to losing their livelihood and even spreading the virus to their family or friends. But if they prioritize their health, they risk falling deeper into economic precarity ([IRC](#) 08/07/2020).

Food Security

COVID-19 measures had multiple impacts on the food security situation of both refugees and host community households in Cox's bazar. The disruption to livelihoods was a significant factor in reducing families purchasing power. This was also compounded by rising prices for some commodities and a lack of access to markets. Refugee communities almost entirely dependent on humanitarian aid reported a reduction in the diversity of food assistance, which was also switched to monthly (instead of twice-monthly) distributions. This increased allocation size combined with movement restrictions made it more difficult for refugee households to transport the food home.

Lack of physical access to market prevented many families from supplementing their diet with fresh vegetables and meat. Many households adopted negative coping mechanisms such as reducing food portions, reducing the number of meals per day, or parents and carers missing meals to provide food to children. Households also reported selling assets or going into debt to purchase food. Typically, female-headed households, children and those whose livelihoods have been most affected (such as day laborers) have been impacted the most.

In the long-term, protracted levels of high food insecurity are expected, particularly among poor households with a knock-on effect on health and nutrition. For the host community, the breakdown of food production and market systems during the crisis could lead to years of decreased agricultural productivity and worsening food and nutrition indicators.

Loss of incomes is driving Food in-security

COVID-19 containment measures have driven a significant increase in food insecurity for refugees and host communities in Cox's Bazar. Following government lockdown measures in March 2020, households in the host community reported immediate impacts such as reduced purchasing power and diminished quality of diet in April 2020 ([WFP](#) 23/09/2020). In May a survey conducted by WFP found that nearly seven out of ten families in the

refugee community and four out of ten families in the host community had an acceptable diet ([WFP 01/07/2020](#)). In June, food affordability continued to be the biggest problem with loss of income reported as the main factor contributing to a poor diet. Purchasing power had dropped by an average of 42 percent in June compared to the pre-crisis period ([WFP 29/07/2020](#), [WFP 23/09/2020](#)). Loss of income was compounded by movement restrictions making it more difficult for refugees to purchase additional food to supplement the quantity and diversity of food they receive through assistance packages ([IOM 31/07/2020](#)).

Refugees have also reported a decrease in the diversity of food assistance, with far less fresh produce due to changes in food distribution modalities and market shortages. Families now receive primarily rice, lentils and oil, and lack the means to buy fresh fruits and vegetables ([IOM 31/07/2020](#), [ACAPS 05/07/2020](#)). As a result, many refugees are concerned about their health and the health of those most vulnerable. Given the reduced access and diversity of food, Rohingya children are at an even higher risk of suffering from acute malnutrition ([ACAPS 05/07/2020](#)).

Food prices and market access are contributing factors to food insecurity

The increasing cost of food was a contributing factor to food insecurity in the first months of the crisis. The cost of a typical food basket increased by about 13 percent between February and April, at a time when food prices were spiking due to lock-down effects on supply chains. Most food commodity supply chains have been disrupted; in particular, poultry, dairy, and fisheries are under stress ([IOM 22/07/2020](#)). Mixed price trends were observed in the markets at the end of March through to May, with increased prices for rice and red lentils, while vegetable oil and garlic remained stable ([IOM 22/07/2020](#)). Increased rice and red lentil prices also reflect increased demand by humanitarian actors mounting relief efforts to support vulnerable people. However, the cost of a basket levelled off from mid-May as supplies improved in the markets ([WFP 30/06/2020](#)). Local camp markets are experiencing increased demand for fresh foods, but supply levels remained low, hence prices for these commodities have risen. This follows the suspension of WFP's fresh food corners and Farmers' Market due to COVID-19 ([WFP 29/07/2020](#)). Movement restrictions (along with the decrease in household purchasing power), has made it more difficult for the refugees to purchase additional food to supplement the quantity and diversity of food they receive through their assistance packages ([IOM 22/07/2020](#)).

Logistical and distribution issues have contributed to food insecurity

Logistic issues caused by COVID-19 containment measures, along with more traditional challenges from monsoon rains, have reduced the amount of food aid agencies able to deliver. The nationwide lockdown following the COVID-19 outbreak has already caused trade flow disruptions for essential food and non-food items ([IOM 22/07/2020](#)). As a containment measure for the spread of COVID-19 in the camps, food distribution frequency was changed from twice-monthly to monthly. This was done to minimize exposure and crowding in the distribution sites that may have facilitated the spread of the virus. Rohingya refugees are now given a full month's entitlement and e-vouchers once a month. The fixed package of food provided is estimated to meet the daily nutritional requirements but there is always a high degree of challenge in providing sufficient food for households and to meet dietary diversity and balanced nutrition ([IOM 22/07/2020](#)). Beneficiaries struggled with increased distances to distribution points, flooded roads, a lack of transport options (to convey supplies back home) and issues with the reduced size and variety of food commodities provided. 82% of the 371 respondents from an IOM survey reported challenges/issues with the provision of food assistance ([IOM 20/09/2020](#)).

Fig 7. Issues and Challenges in Food Distribution (extracted from the COVID-19 NPM-IVR Needs Assessment conducted by IOM July/August 2020)

Key issues with the food being distributed (178 respondents):

- Amount of food is not sufficient (84%)
- Missing food items (46%)
- Received less than supposed to (food items are now pre-packaged) (42%)

Top three issues accessing distribution points (107 responses):

- Takes long time to reach the distribution point (89%)
- Longer waiting time than usual (65%)
- Discrimination at distribution center (8%)

Top five challenges while collecting food assistance (371 responses):

- Issues with the food being distributed (59%)
- Issues accessing distribution points (35%)
- Issues with transporting the food back to shelter (29%)
- Issues with the frequency of distribution (22%)
- Issues with SCOPE card/documentation (8%)

Top three challenges transporting food back (88 responses):

- Issues with porter service (60%)
- Unable to carry items home (53%)
- Roads are inaccessible during monsoon (44%)

Adoption of negative coping mechanisms

With a significant loss of purchasing power, restricted access to markets and a reduction in the availability of certain food commodities, households have adopted a variety of different negative coping strategies. Even so many families reported not being able to manage an acceptable diet.

Use of negative coping mechanisms was already widespread **before** the COVID-19 pandemic with almost 80% of Rohingya households used negative coping mechanisms to meet their basic needs ([ACAPS 05/07/2020](#)). The 2019 Joint Multi-Sector Needs Assessment (J-MSNA) found at least one-third of households reporting "selling, sharing and/or exchanging food ration in the previous 30 days" ([IOM 20/09/2020](#)).

Rohingya refugees continue to resort to negative coping mechanisms related to food insecurity which can contribute to health risks or lead to a cycle of debt. These include borrowing food, relying on help from friends and relatives, eating smaller portions and reducing the number of meals eaten per day ([IOM 31/07/2020](#)). In the host community 9 out of 10 households reported resorting to livelihood-based coping strategies to meet both food and non-food needs with spending savings and buying food on credit being the most prevalent. Interestingly there was a slight variance on the type of strategy adopted depending on the gender of household head, with relatively more male-headed households involved in spending savings and the sale of jewelry, while more female-head households used buying food on credit, reducing non-food expenses and selling household goods ([WFP 07/09/2020](#)).

Vulnerable groups

Expenditure on food has dropped significantly highlighting how the crisis has impacted poor populations who characteristically spend most of their income on food. Expenditure on food dropped by 48 percent compared to pre-crisis levels, clearly illustrating the impact of income losses on household spending patterns. The drop-in food expenditure is somewhat proportionate to the overall drop in income levels. These findings potentially point to a latent nature of economic vulnerability, which characterize the poor populations: most of their income is spent on food ([WFP](#) 07/09/2020). Typically, female-headed households, and those whose livelihoods have been most affected (such as day laborers) have been impacted the most.

The loss of livelihoods, coupled with decreased access to the local markets, has disrupted economic activity and increased the support needs of Bangladeshis in the District and could lead to longer term food insecurity. This is especially true for the most marginalized and vulnerable, such as persons living with disabilities, female headed households and the elderly. Reduction in the number of meals consumed was relatively higher among female-head households with children under five, lactating and pregnant women ([IOM](#) 22/07/2020, [WFP](#) 07/09/2020, [World Vision](#) 09/07/2020). Protracted levels of high food insecurity are expected, particularly among poor households. The breakdown of food production and market systems during the crisis could lead to years of decreased agricultural productivity and worsening of food and nutrition indicators [for host communities] ([IOM](#) 22/07/2020).

Protection

In already cramped conditions with the camps, the imposition of containment measures and movement restrictions has led to an upsurge in protection issues. One factor is the impact of livelihoods and food in-security. Reduction in income, lack of access to markets and some price increases have contributed to increased stress within households. This has compounded the rise in violence and crime.

Containment measures have also resulted in a reduction in humanitarian activities making it more difficult to access protection services. Women and children are especially vulnerable with children no longer accessing their peer groups due to education centers remaining closed and movement restrictions preventing many recreational activities. Communities and agencies are both reporting an increase in violence in the home and against children, there are higher rates of early marriage and child labor and a significantly increased risk of SGBV.

Drivers linked to the pandemic causing a rise in Protection issues in line with global trends

The impact of containment measures is causing stress, reducing access to services and livelihoods, and driving an increase in protection issues. Similar to trends seen globally for refugees, the pandemic situation in Cox's Bazar is negatively affecting the overall protection environment with the Rohingya refugee population, the host community, and protection actors operating in Cox's Bazar all reporting an increase in protection issues in the camps and surrounding areas ([UNHCR](#) 13/09/2020, [ACAPS](#) 05/07/2020). The emergence of COVID-19 has exacerbated pre-existing protection risks for refugees and host communities alike due to the socio-economic impact of the virus. The disruption of protection services, the closure of community centers, child-friendly spaces, and learning centers exposes individuals to additional social and legal protection risks that require tailored responses in the context of the pandemic ([IOM](#) 22/07/2020).

The security situation is a concern with inter and intra-communal disputes and increased levels of criminality bringing heightened risks of SGBV and people trafficking. The pandemic has increased discrimination, stigmatization, hostility and incitement against returning migrants, the Rohingya refugees, and people from districts with high infection rates ([UNCT](#) 16/09/2020). Tensions and insecurity may also arise as the Rohingya refugees and humanitarians may be blamed for spreading the virus because of the unhygienic and overcrowded conditions in the camps. An increase in hate speech, racism and stigmatization is already evident in local reporting and social media, and in the rise in crime and attacks ([IOM](#) 22/07/2020). Also of concern is how the socio-economic impact of COVID 19 is having a negative impact on security; with petty crimes, inter and intra-communal disputes rising, criminal groups expanding their spheres of control with relative impunity, and heightened risks of traffickers and smugglers preying on vulnerable refugees and host communities alike. SGBV, child protection cases and cases of intimate partner violence have also been exacerbated by the pandemic ([UNHCR](#) 11/08/2020).

Sexual and Gender Based Violence

COVID-19 Containment measures are both a driver for increased sexual and gender-based violence (SGBV) and a barrier to the provision of protection services with female-headed households and households with many dependents being particularly vulnerable. Sexual and gender-based violence has long posed a problem throughout the Rohingya camps in Cox's Bazar and continues to persist in the camps, especially intimate partner violence ([Independent Diplomat](#) 27/07/2020, [UNHCR](#) 10/08/2020). Domestic violence has become a serious issue as anxiety over food shortages and residing in close quarters boils into frustration and aggression. Increased restrictions, loss of livelihoods, suspension of educational activities, and increased criminality, among others, have compounded SGBV risks. The lock-down is exacerbating such problems by both forcing a shut-down of child and women-friendly spaces and impeding access to protection services by survivors of intimate partner violence due to the prolonged presence of their spouses at home ([Independent Diplomat](#) 27/07/2020, [UNHCR](#) 11/08/2020).

The overall situation is having a severe impact on the health and well-being of women and girls particularly, child marriage continues to be of concern and there is also a worrying rise in girls talking about suicide ([UNHCR](#) 10/08/2020). The COVID-19 zonal lock-down during June further limited access to camps for many partners' staff who faced transport challenges to reach the camps, causing delays in delivery of important GBV services. Due to movement restrictions and heavy rainfall, fewer people are attending GBV awareness sessions in some camps ([ISCG](#) 27/07/2020). Female-headed households, and households with many dependents are among the most vulnerable in the camps. Their risk of exposure to GBV is also increased due to the need to stay indoors, as well as the reduced presence of case workers to receive and address GBV incidents ([IOM](#) 22/07/2020).

Children and youth at increased risk

Children and youth are facing increasing protection risks due to the socio-economic impact of COVID-19 containment measures, at the same time protection services have been inhibited by new working restrictions. In Cox's Bazar, adolescents and youth already face very high levels of domestic and intimate partner violence and violence against children is increasing with children witnessing or experiencing violence in the home due to heightened stress, reduced resources and lack of support services. This can have enormous negative impacts on children's mental health and well-being. Children particularly girls, face greater risk of sexual and gender-based violence, including rape and domestic abuse due to limited oversight and reduced avenues to report during

COVID-19 ([UNICEF](#) 17/09/2020, [IOM](#), 22/07/2020). As many families are being pushed into extreme poverty and hunger by COVID-19, parents are increasingly unable to feed their families. There has been an increase in child labor in the camps, for example, with children engaged in casual work to earn money for their families or completing other tasks such as collecting food and gas at distribution points on behalf of their families. ([UNICEF](#) 17/09/2020).

With access to support from peer groups cut there has been a rise in children adopting negative coping mechanisms. The increases in risk and rising protection issues have caused psychological distress which has been further compounded by the absence of alternative recreational and learning activities (following the closure of all community centers, learning facilities and child-friendly spaces). This has resulted in children and adolescents developing a range of negative coping mechanisms ([UNHCR](#) 11/08/2020). UNHCR and its partners are following up on 5,000 child protection cases. The number of cases increased significantly after the COVID-19 response started. It is a serious indication of the impact and consequences of the pandemic on the refugee community ([UNHCR](#), 10/08/2020).

Education

The education system for the Rohingya was already challenging with many children having limited access to education in Myanmar before fleeing to Cox's Bazar. Education services within the camp were not aligned with formal systems of either the host country or country of origin but were provided through informal education centers. Recent efforts to bring in a formal system with accreditation have been put on hold due to the pandemic.

Education Centers were closed in mid-March along with a nationwide closure of education establishments. Efforts have been made to provide education through home learning initiatives, however without access to the internet this has mainly relied on learning packs produced locally and provided to caregivers. The initiative has faced challenges with the teachers and education volunteers lacking the skills and training to provide relevant quality materials; in addition the low educational level of parents and caregivers has limited their ability to use the packs and deliver effective home schooling.

The lack of access to education centers has also had a significant negative impact on children's well-being and development. Lack of access to their peer group and to normal recreational activities due to lockdown restrictions is having an adverse effect on children's mental well-being. There are also increased reports of child marriage, child labor and domestic violence.

Schools across Bangladesh remain closed until at least October 3, with no information as to when learning centers in Cox's Bazar will reopen. There is a real risk that significant numbers of children will not return to school, especially girls, and those that do will have missed out on half a year of schooling.

Education Centers are still closed

Education centers across Bangladesh were closed in March 2020 and approximately 325,000 refugee children have been without access to learning facilities since that time; host community children have been similarly affected. Nearly 6,000 learning centers with the camps remain closed for the foreseeable future with no timetable for their reopening ([UNICEF](#) 05/08/2020). Efforts have been made to support many Rohingya children to continue

their learning from home with small-scale support from Rohingya volunteers and teachers and the provision of stationery and learning materials ([ACAPS](#) 05/07/2020). However these efforts have been challenging as education was classified as a "non-essential" intervention by the Office of the Refugee Relief and Repatriation Commissioner ([RRRC](#), 08/04/2020).

The lack of access to education (and other essential services delivered through education centers), and the limiting of recreational activities is having a heavy impact on children's mental and emotional wellbeing. Children, adolescents, and youth no longer have access to essential services delivered through education and skills programs, such as food (through school feeding programs), sexual reproductive health awareness and services, and child protection services, all of which have been temporarily suspended ([ACAPS](#) 05/07/2020). The closure of learning centers, child friendly centers and other venues for children has resulted in further exposure of children to protection risks, giving rise to increased behavioral challenges and the adoption of negative coping mechanisms ([UNCHR](#) 10/08/2020). Without access to learning facilities – and in isolation – there is little scope to provide clear communication messaging, psychosocial support to both students and educators, as well as meet their social-emotional learning needs or provide life skills education. ([UNICEF](#) 05/08/2020).

Distance learning is facing challenges

Distance learning is facing significant challenges. Rohingya Children are being helped to learn at home by engaging parents and caregivers to support learning and through the provision of workbooks and visual aids. Rohingya volunteer teachers have played a central role while at the same time delivering COVID-19 health and hygiene messages ([UNICEF](#) 25/08/2020). However, teachers, parents and caregivers lack the tools and strategies to home-school their children without structured guidance, while also ensuring children's safety, security, and physical mental well-being. The majority of Rohingya caregivers/parents have low educational attainment, lack experience in distance schooling, and will therefore struggle to support their children's learning ([UNICEF](#) 05/08/2020). In the Rohingya camps in Cox's Bazar, mobile data and internet use are banned², without access to the internet in the camps, many technology-based solutions are not viable. In addition, teachers and education facilitators, both from the host and refugee communities, are disconnected from their students and from regular continuous professional development activities, as Education Partners have suspended all training and coaching programs even though teachers desperately need support in self-learning to equip themselves with the new skills needed for remote teaching ([IRC](#) 25/08/2020, [UNICEF](#) 05/08/2020).

Children are being heavily impacted by containment measures

Lack of education development is only part of the picture when considering how heavily containment measures have affected the children in Cox's Bazar. The lack of access to education and recreational activities combined with long periods at home in cramped family conditions is taking a heavy toll on children's well-being as well as exposing them to increased protection risks and harmful coping mechanisms. Without access to their usual peer networks or community-based learning opportunities, girls will become more vulnerable to exclusion more isolated and more susceptible to gender-based violence, and child labor is particularly a risk for boys. Children are also more likely to be affected by increased levels of domestic violence as caregivers and parents are spending more time at home, exacerbating the existing problems in overcrowded camps. Children who experience learning and/or physical challenges will face additional difficulties through further lack of support, frustration at home, and the

² [3G and 4G mobile networks and internet were restored](#) on at the end of August after a ban on mobile phones services introduced in September 2019

discontinuation of positive outlets ([UNICEF](#) 05/08/2020). Continued school closures and movement restrictions can lead to increased levels of emotional disturbance and anxiety among adolescents and youth, and in turn lead to higher incidences of violence in the home, unhealthy coping strategies, and suicidal ideation ([IOM](#) 22/07/2020).

Long-term consequences

Continued lack of access to quality education services is significantly harming the development of refugee children and as the closure continues this will likely lead to increased school dropout and engagement in negative coping mechanism such as child labor and early marriage. Even prior to the COVID-19 pandemic, Rohingya refugee children were generally neither enrolling nor attending education programs consistently, with many children already having their educational development interrupted by displacement from Myanmar. The current informal education programming is also limited by the lack of any form of accreditation or qualifications for graduating students. Globally the post-lockdown forecast for refugee girls is particularly grim; based on an analysis of UNHCR data on school enrolment, the Malala Fund has estimated that half of all refugee girls in school will not return when classrooms reopen in September ([UNHCR](#), 2020/09/02).

Information gaps and challenges

The analysis in this report is based on publicly available documents. The intention is to integrate data directly from field sources in future iterations. The lack of granular field data means it has not been possible to quantify much of the impact within this report.

COVID-19 Infection rate

Robust data is available on the number of COVID-19 cases, people in isolation, COVID-19 tests carried out and deaths attributed to COVID-19. However not all those with COVID-19 have symptoms and not everyone with COVID-19 will be tested (this is true globally). This is especially problematic within Cox's Bazar where the reticence of the population to testing has been highlighted within the analysis. Therefore, any discussion on the prevalence of COVID-19 and likely progression of the infection rate has to be taken with a degree of caution as the actual situation on the ground will be different to that shown in the data.

Livelihoods and Food Security

The impact of the crisis on livelihoods has been considerable, but it is harder to understand the medium to long term implications. We have some data, for example a lot of information on the prevalence of negative coping mechanisms and estimates on those now falling below certain vulnerability or poverty thresholds, but many of these numbers are based on estimates. Therefore, current data is needed to understand if the estimates were correct and determine the current numbers of jobless and families below the poverty line.

Self-reporting data has given us a better reflection of the current situation, with the Cox's Bazar urban vulnerability assessment (WFP) and COVID-19 NPM-IVR Needs Assessment (IOM) providing good examples:

- Spending savings and buying food on credit were the widely adopted strategies, reported by 63 and 44 percent of households, respectively ([WFP](#) 07/09/2020).

- Top three issues accessing distribution points (107 responses): Takes long time to reach the distribution point (89%), Longer waiting time than usual (65%), Discrimination at distribution center (8%) ([IOM 20/09/2020](#)).

However, data on the food situation for families is still not clear. Reporting of issues with food distribution, does not show how many families are missing meals. We also need to understand the situation overtime (NPM-IVR has now completed 2 rounds); is the situation returning to pre-crisis levels or are the changes permanent? Finally, self-reporting on its own comes with a degree of bias depending who is asked and what might influence their answer (with over-reporting needs in the expectation of assistance a common issue in humanitarian settings).

Impact of Restrictions on Humanitarian Activities

Again, it is difficult to quantify the effect of restrictions on humanitarian programming. Data has not been sourced to show exactly what programming did not take place due to restrictions, or what programming had reduced effectiveness. Normal program monitoring (using tools such as the 3W) could give some idea of those reached, but this would have to be compared to the expected beneficiary reach. Also, it is probable that additional costs have been incurred (for example to meet safety requirements) and this will also reduce beneficiary reach unless extra funding is acquired.

Protection and Education

Education outcomes are usually measured in terms of enrolment, attendance, progression and transition rates, exam results and drop-out rates. Until schools and education centers reopen it will be difficult to calculate what impact the crisis has had on education. There is little data on how effective distance learning and home schooling has been with limited qualitative data listing only the issues and limitations.

For Protection there are also gaps in understanding the actual numbers and prevalence of issues. Access to the referral mechanisms has been negatively impacted by the crisis so this could mean less cases make it into the referral caseload, and there are frequent references to increased child marriage and child labor but no quantitative data was available. Protection data is of course extremely sensitive and therefore much will not be made publicly available for safeguarding reasons. A clearer and more quantified analysis of the Protection sector will require access to the relevant datasets.

Annex – About this report

The OFDA COVID-19 support project is currently implemented by IMMAP and DFS in six countries: DRC, Burkina Faso, Nigeria, Bangladesh, Syria and Colombia. The project duration is twelve months and aims at strengthening assessment and analysis capacities in countries affected by humanitarian crises and the COVID-19 pandemic. The main deliverables of the project are a monthly country level situation analysis, including an analysis of main concerns, unmet needs and information gaps within and across humanitarian sectors.

The first phase of the project (August-November 2020) is focused on building a comprehensive repository of available secondary data in the DEEP platform; build country networks and provide a regular analysis of unmet needs as well as the operational environment within which humanitarian actors operate. As the repository builds up, the analysis provided each month will become more complete and robust.

Methodology. To guide data collation and analysis, IMMAP and DFS designed a comprehensive Analytical Framework allowing to address specific strategic information needs of UN agencies, INGOs, LNGOs, clusters and HCTs at country level. It is essentially a methodological toolbox used by IMMAP/DFS Analysts and Information Management Officers during the monthly analysis cycle. The Analytical Framework:

- Provides with the entire suite of tools required to develop and derive quality and credible situation analysis;
- Integrates the best practices and analytical standards developed in recent years for humanitarian analysis;
- Offers end users with an audit trail on the amount of evidence available, how data was processed and conclusions reached;

The two most important tools used throughout the process are the Secondary Data Analysis Framework (SDAF) and the Analysis Workflow.

The Secondary Data Analysis Framework was designed to be compatible with other needs assessment frameworks currently in use in humanitarian crises (Colombia, Nigeria, Bangladesh) or developed at the global level (JIAF, GIMAC, MIRA). It focuses on assessing critical dimensions of a humanitarian crisis and facilitates an understanding of both unmet needs, their consequences and the overall context within which humanitarian needs have developed and humanitarian actors are intervening. A graphic representation of the SDAF is available in figure 8.

On a daily basis, IMMAP/DFS Analysts and Information Management Officers collate and structure available information in the DEEP Platform. Each piece of information is tagged based on the pillars and sub-pillars of the SDAF. In addition, all the captured information receives additional tags, allowing to break down further results based on different categories of interest, as follows:

- 1) Source publisher and author(s) of the information;
- 2) Date of publication/data collection of the information and URL (if available);
- 3) Pillar/sub-pillar of the analysis framework the information belongs to;
- 4) Sector/sub-sectors the information relates to;

1. Context					
Political Legal & policy	Security Economics	Socio cultural Infrastructure	Demographic Environment		
2. Events/Shock/Ongoing Conditions					
Type & Characteristics	Drivers & Aggravating factors	Mitigating factors			
3.COVID-19 Containment related measures					
Physical distancing	Movement restrictions	Public Health measures	Lockdowns		
4. Displacement					
Type #, Mvmt	Push factors	Pull factors	Intentions	Local integration	
5. Casualties					
Injured	Missing		Dead		
6. Humanitarian Access					
Access of affected population to assistance	Access of relief actors to the affected population	Security / physical constraints	People facing humanitarian access constraints		
7. Communication and Information					
Information channels and means		Information challenges		Information Needs	
12. Capacities / Response	National/local actors			Cross Protection WASH Education Food Sec. Health Shelter Livelihood Nutrition	
	International actors				
	Government & local authorities				
	Priority interventions (pop.)				
11.Priorities	Priority needs (Hum.)				
	Priority needs (pop)				
	8. Impact	Impact on people			
10.At Risk	People at risk/vulnerable				
	Physical / mental wellbeing				
	Coping mechanisms				
9.Humanitarian Conditions	Living standards				
	Impact on services and systems				
	Analytical Outputs	Crisis impact:	Humanitarian profile, Affected people	Severity of humanitarian conditions: PIN by severity class; People at risk	Number of people at risk
Gaps in response: PIN Reached PIN Covered PIN not reached					

Country: Bangladesh Period: June to September 2020 # Update: 01 Report Status: Public

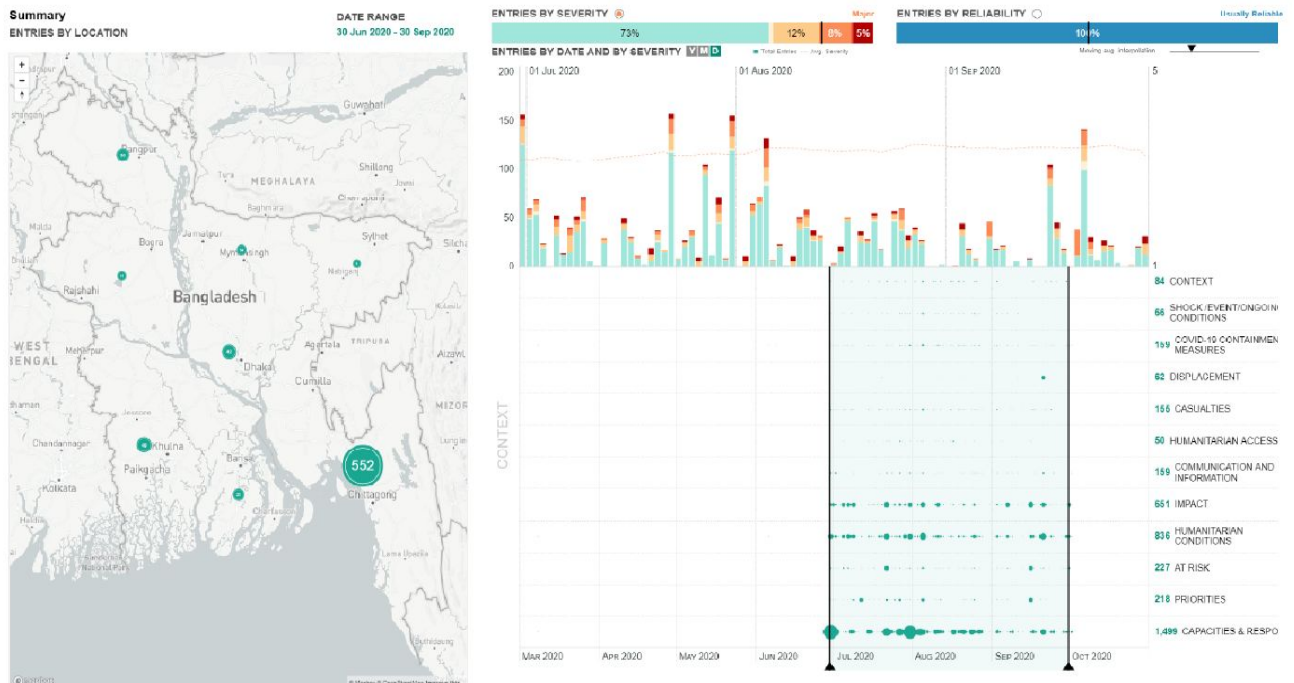
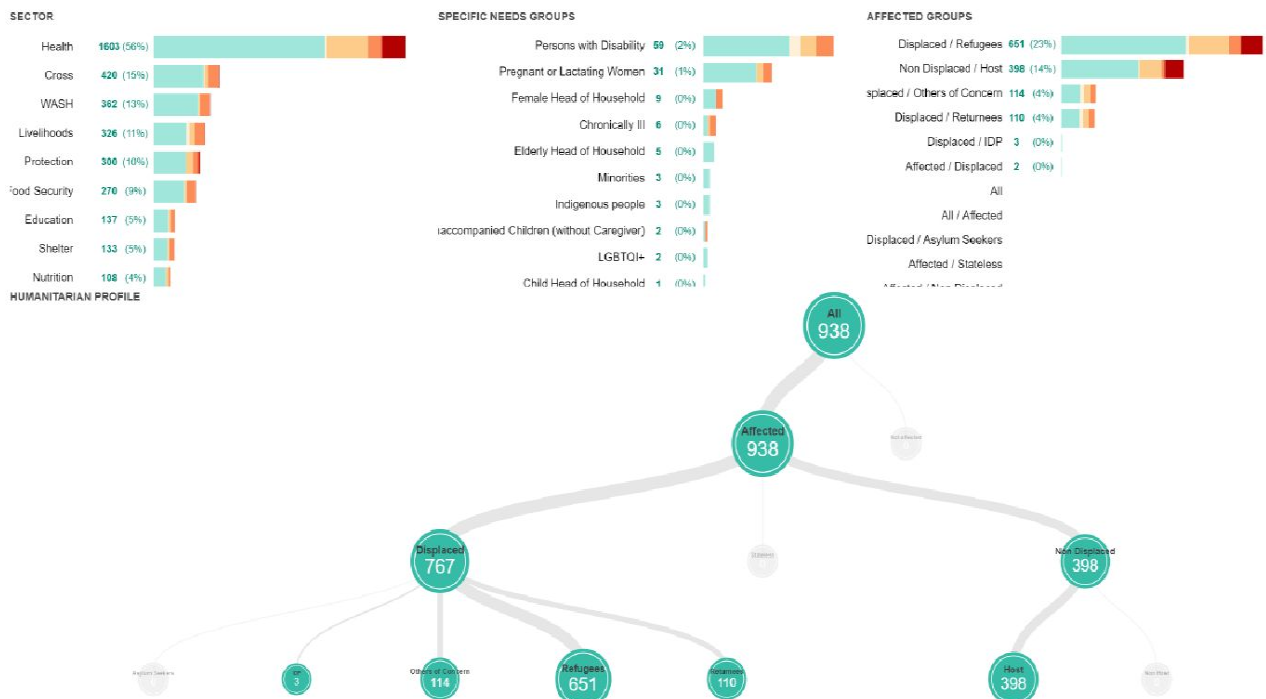
Figure 9: Documents by Location, Timeline and Primary Categories (Analytical Framework)

Figure 10: Documents and Entries by Sector and Affected Group


Figure 11: Entries by Sector and sub-Categories (Analytical Framework)

SECTORAL FRAMEWORK	# of Entries	median severity	CROSS	FOOD SECURITY LIVELIHOODS	HEALTH	NUTRITION	WASH	SHELTER	EDUCATION	PROTECTION	AGRICULTURE	LOGISTICS
IMPACT 724	Drivers/Aggravating Factors	339										
	Impact on People	254										
	Impact on System & Services	276										
	Number of People Affected	16										
HUMANITARIAN CONDITIONS 89	Living Standards	292										
	Coping Mechanisms	66										
	Physical & mental wellbeing	470										
	Number of People in Need	4										
AT RISK 218	People at risk / Vulnerable	218										
PRIORITIES 208	Priority Needs (Pop)	29										
	Priority Needs (Staff)	14										
	Priority Interventions (Pop)	4										
	Priority Interventions (Staff)	168										
CAPACITIES & RESPONSE 812	Government & Local Authorities	153										
	National & Local Actors	353										
	International	778										
	Number of People Reached	619										

Analysis Workflow. IMMAP/DFS analysis workflow builds on a series of activities and analytical questions specifically tailored to mitigate the impact and influence of cognitive biases on the quality of the conclusions. The IMMAP/DFS workflow includes 50 steps. As the project is kicking off, it is acknowledged that the implementation of all the steps will be progressive. For this round of analysis, several structured analytical techniques were implemented throughout the process to ensure quality results.

- The ACAPS Analysis Canvas was used to design and plan for the September product. The Canvas support Analysts in tailoring their analytical approach and products to specific information needs, research questions or information needs.
- The Analysis Framework was piloted and definitions and instructions set to guide the selection of relevant information as well as the accuracy of the tagging. A review workshop will be organized in October 2020 to review pillars and sub pillars and adapt if necessary.
- An adapted interpretation sheet was designed to process the available information for each SDAF's pillar and sub pillar in a systematic and transparent way. The Interpretation sheet is a tool designed so IMMAP/DFS analysts can bring all the available evidence on a particular topic together, judge the amount and quality of data available and derive analytical judgments and main findings in a transparent and auditable way.
- Information gaps and limitations (either in the data or the analysis) were identified. Strategies have been designed to address those gaps in the next round of analysis.

The analysis workflow is provided overleaf (Figure 12).

Figure 12 – IMMAP/DFS Analysis Workflow

IMMAP/DFS Analysis Workflow – September 2020					
	1.Design & Planning	2.Data collation & collection	3.Exploration & Preparation of Data	4.Analysis & Sense Making	Sharing & Learning
Main activities	Definitions of audience, objectives and scope of the analysis,	Identification of 195 relevant documents (articles, reports) from 53 sources	Categorization of the available secondary data (2873 excerpts)	Description (summary of evidence by pillar / sub pillar of the framework)	Report drafting, charting and mapping
	Key questions to be answered, analysis context, Analysis Framework	Identification of relevant needs assessments	Assessment registry (20 Needs assessment reports)	Explanations (Identification of contributing factors)	Editing and graphic design
	Definition of collaboration needs, confidentiality and sharing agreements	Data protection & safety measures, storage	Additional tags	Interpretation (priority setting, uncertainty, analytical writing)	Dissemination and sharing
	Agreement on end product(s), mock up and templates, dissemination of products	Interviews with 3 key stakeholders	Information gaps identification	Information gaps & limitations	Lessons learnt workshop, recommendations for next round
Tools	Analysis Framework Analysis Canvas Data sharing agreements Report template	SDR folder Naming convention	DEEP (SDAF) DEEP (Assessment registry) Coding scheme	Interpretation sheet	Revised report template Analytical writing guidance Lessons learnt template

Contact

For further information on the project you can contact:

Bangladesh focal point: **Alex Nwoko** | anwoko@immap.org

Global project manager: **Francesco Baldo** | fbaldo@immap.org